INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Nivolumab, ipilimumab, paclitaxel and carboplatin

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
42 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00712
Version 3a

Treatment stages

01 **Cycle 1: two chemotherapy administrations**
Cycle 1: two chemotherapy administrations

02 **Immune continuation**
Immune continuation

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Follow your oncology team's emergency instructions for fever, shivering or sudden illness, including with normal temperature. Report new breathlessness/cough, diarrhoea or abdominal pain, reduced urine, severe rash, unusual weakness, headache or marked thirst promptly. Severe chest pain, collapse, confusion or major breathing difficulty needs emergency care.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

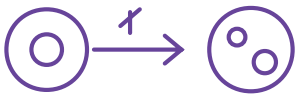
Three medicines. Different jobs.

This treatment aims to control metastatic squamous non-small-cell lung cancer, help symptoms and extend life. It is usually given without an expectation of cure.

IMMUNOTHERAPY

Nivolumab

Opdivo



Blocks the PD-1 immune checkpoint

Nivolumab binds PD-1 on T cells. It prevents signals that can reduce the immune attack on cancer. This can also let immune cells inflame healthy tissues. It is infused every 21 days in this plan.

T cell → PD-1 blocked → Immune response

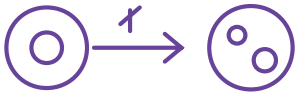
What to watch for

New breathing, bowel, skin, hormone or other symptoms need prompt advice during or after treatment.

IMMUNOTHERAPY

Ipilimumab

Yervoy



Blocks the CTLA-4 immune checkpoint

Ipilimumab blocks CTLA-4, another control on T-cell activity. It works alongside nivolumab through a different immune checkpoint. It is given every 42 days, so it is absent from each Day 22 visit. Healthy organs can also become inflamed.

T cell → CTLA-4 blocked → Immune activity

What to watch for

Report diarrhoea, tummy pain, rash, severe tiredness, headache or other new symptoms promptly. Keep your immune alert card.

CHEMOTHERAPY

Paclitaxel

Taxane chemotherapy



Disrupts microtubules and cell division

Paclitaxel stabilises microtubules, structures cells use during division. It can slow cancer growth but also harm healthy nerves, blood-forming cells and hair follicles. There are two planned doses.

Microtubules → Division disrupted → Cancer cells affected

What to watch for

Tell the nurse immediately about infusion symptoms. Report numbness, tingling, fever, painful muscles, mouth sores or diarrhoea.

CHEMOTHERAPY

Carboplatin

Platinum chemotherapy



Damages deoxyribonucleic acid (the cell’s genetic material) (DNA)

Carboplatin forms links in DNA that hinder copying and repair. Healthy tissues can also be harmed. Its dose depends on target exposure (AUC) and measured or estimated kidney function; AUC is not a fixed number of milligrams. There are two planned doses in this regimen.

DNA → Platinum links → Copying disrupted

What to watch for

Report fever, bleeding, nausea, hearing changes or numbness. Tell the nurse immediately about possible allergy during the infusion.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment is offered

What does the diagnosis mean?

Squamous non-small-cell lung cancer (NSCLC) is a lung-cancer histology. Metastatic means it has spread; the team confirms EGFR/ALK results and first-line suitability.

What are the alternatives?

Ask about other systemic options, trials and symptom-focused care in the context of your cancer, health and priorities.

Why this treatment is offered

The combined strategy improved survival compared with chemotherapy in the trial population, with some durable responses. Benefits and harms vary between people.

Trial results compare groups and cannot predict your personal benefit. Discuss how the study population relates to your cancer and health.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Induction → review → maintenance

Treatment stages

01 **Cycle 1: two chemotherapy administrations**
Cycle 1: two chemotherapy administrations

02 **Immune continuation**
Immune continuation

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

CYCLE 1: TWO CHEMOTHERAPY ADMINISTRATIONS

Cycle 1: two chemotherapy administrations

D1 D22

One 42-day cycle, chemotherapy Days 1 and 22 only. Cycle 1 lasts 42 days: nivolumab, ipilimumab, paclitaxel and carboplatin on Day 1; nivolumab, paclitaxel and carboplatin on Day 22. These are the only two planned chemotherapy administrations. From Cycle 2, nivolumab is given on Days 1 and 22 and ipilimumab on Day 1.

IMMUNE CONTINUATION

Immune continuation

D1 D22

Until progression, unacceptable toxicity or up to 24 months without progression. Cycle 1 lasts 42 days: nivolumab, ipilimumab, paclitaxel and carboplatin on Day 1; nivolumab, paclitaxel and carboplatin on Day 22. These are the only two planned chemotherapy administrations. From Cycle 2, nivolumab is given on Days 1 and 22 and ipilimumab on Day 1.

Maintenance means ongoing treatment, not treatment without side effects. Your plan can change after discussion.

BASELINE → TREATMENT → FOLLOW-UP

Checks throughout treatment

Relative stages; no dates need to be entered. Ask who arranges and reviews each check.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00712 v3a · official regimen

THE WHOLE TREATMENT JOURNEY

Nivolumab, ipilimumab, paclitaxel and carboplatin: treatment and checks

First-line metastatic squamous non-small-cell lung cancer (NSCLC) in adults without sensitising EGFR mutation or ALK translocation.

Cycle 1 lasts 42 days: nivolumab, ipilimumab, paclitaxel and carboplatin on Day 1; nivolumab, paclitaxel and carboplatin on Day 22. These are the only two planned chemotherapy administrations. From Cycle 2, nivolumab is given on Days 1 and 22 and ipilimumab on Day 1. Continue while appropriate until progression, unacceptable toxicity or up to 24 months without progression. Your team reviews the plan and may delay or stop treatment.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Cycle 1 · Day 1	Cycle 1 · Day 22	Later cycles · Day 1	Later cycles · Day 22	Completion / follow-up
Treatment NCCP 00712	Confirm indication and plan	All four medicines	Nivolumab + final paclitaxel/ carboplatin	Nivolumab + ipilimumab	Nivolumab alone	Agree stop plan and continuing care
Blood count, kidney and liver NCCP 00712	Baseline results	Review before treatment	Review before treatment	Review before every cycle	Review before treatment	Follow unresolved toxicity
Hormones and immune symptoms NCCP 00712	Glucose, thyroid function tests (TFT); hepatitis screen; cortisol if indicated	Glucose/ TFT before cycle and symptom review	Review symptoms; extra tests if needed	Glucose/ TFT before cycle and symptom review	Review symptoms; extra tests if needed	Vigilance at least five months after last dose

Care / check	Before starting	Cycle 1 · Day 1	Cycle 1 · Day 22	Later cycles · Day 1	Later cycles · Day 22	Completion / follow-up
Support medicines NCCP 00712	Paclitaxel premedication includes corticosteroid and antihistamine treatment, with H2-blocker decisions made by the team. The source allows consultant-directed changes; ask for a written plan for each of the two chemotherapy visits.	Chemotherapy premedication/ antiemetics	Repeat prescribed chemotherapy support	Low-risk support; no chemotherapy prophylaxis	Minimal-risk support; no routine prophylaxis	Confirm medicines and steroid taper if used
Response and preferences NCCP 00712	Discuss goals and alternatives	Ask about symptoms and priorities	Review treatment experience	Consultant-directed scans and benefit review	Report new concerns between visits	Agree responsible team and symptom support

What each check is for

1 Baseline

Before first infusion

Confirm exact eligibility

Ask / **act:** Review results, consent, medicines and contacts.

2 Day 22

Three weeks after Day 1

Different active medicines

Ask / **act:** No ipilimumab on Day 22; confirm support plan.

3 **Continuation**

Later 42-day cycles

Review benefit and toxicity

Ask / **act:** Apply joint immune hold rules and review duration.

4 **End of treatment**

At progression/toxicity or up to 24 months

Late immune effects can occur

Ask / **act:** Document follow-up and urgent contacts.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Visits and duration

STEP 2

Time in the unit

Nivolumab over 30 minutes, then ipilimumab over 30 minutes on Day 1 only. Paclitaxel over three hours, then carboplatin over 30 minutes on both Cycle 1 treatment days. Ipilimumab vital signs are checked every 30 minutes during infusion and for an hour afterwards. Blood tests, preparation and observation add to time in the unit.

STEP 3

Medicines and monitoring

Blood count, kidney and liver tests before each treatment; glucose and thyroid tests before each 42-day cycle. Cortisol and neuropathy checks when indicated. Paclitaxel premedication includes corticosteroid and antihistamine treatment, with H2-blocker decisions made by the team. The source allows consultant-directed changes; ask for a written plan for each of the two chemotherapy visits.

STEP 4

Transport and support

Arrange travel and help if needed. Keep urgent contacts and alert information with you.

Before the first visit

Your usual health

Describe usual bowel movements, breathing, energy and skin; report autoimmune illness, infection, nerve symptoms and every medicine or supplement.

Practical preparation

Arrange blood tests, transport and support at home. Keep treatment names, urgent contacts and your immune alert card available.

Checks before and during treatment

Before starting

Baseline full blood count (FBC), renal/liver profile, glucose and thyroid function tests (TFTs); hepatitis B surface antigen and hepatitis C RNA. GFR for carboplatin; audiology/creatinine clearance (CrCl) and neuropathy assessment when indicated.

New symptoms

Extra tests may be needed between visits. Routine blood results alone cannot exclude every immune effect.

Response

Scans follow the consultant plan; discuss benefit, uncertainty and what would lead to changing treatment.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 42: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Day 1 visit	 **Day 1** Includes nivolumab and ipilimumab; chemotherapy in Cycle 1 only
Day 22 visit	 **Day 22** Nivolumab; no ipilimumab; final chemotherapy in Cycle 1
Immune symptoms	 **Days 1–42** Any day, including after treatment stops
Chemotherapy effects vary	 **Days 1–42** After Cycle 1 doses: sickness, low counts, fatigue, nerve or other effects; no guaranteed recovery day

Treatment schedule: Cycle 1 lasts 42 days: nivolumab, ipilimumab, paclitaxel and carboplatin on Day 1; nivolumab, paclitaxel and carboplatin on Day 22. These are the only two planned chemotherapy administrations. From Cycle 2, nivolumab is given on Days 1 and 22 and ipilimumab on Day 1.

Duration: Continue while appropriate until progression, unacceptable toxicity or up to 24 months without progression. Your team reviews the plan and may delay or stop treatment.

No guaranteed symptom-free days: Side effects vary and immune symptoms can start after treatment stops.

After treatment: Both immune medicines can inflame healthy organs during treatment or after it stops. Keep your immune alert card and report new symptoms promptly, including for at least five months after the last dose.

> **Why these bands stay broad:** 42-day reference cycle with different Day 1 and Day 22 medicines. Symptom bands are not a prediction of recovery. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.

There are no guaranteed "safe days". Both immune medicines can inflame healthy organs during treatment or after it stops. Keep your immune alert card and report new symptoms promptly, including for at least five months after the last dose.

Between-treatment notes

Day 1: Day 1 visit

Nivolumab and ipilimumab are due. Paclitaxel and carboplatin are also due in Cycle 1 only. Report symptoms and confirm the current prescription.

Day 2–21: Between visits

Follow your prescribed support plan and report new symptoms promptly. Immune effects can occur on any day. Keep the oncology contact instructions and immune alert card accessible.

Day 22: Day 22 · no ipilimumab

Nivolumab is due. In Cycle 1 this also includes the second and final chemotherapy administration. Confirm active medicines and the right support plan.

Day 23–42: Before the next cycle

Continue symptom reporting. Later Day 1 visits again include both immune medicines if prescribed. Confirm the next appointment and blood tests; do not wait for it if unwell.

Support around the medicines

1 Cycle 1 Days 1 and 22 · chemotherapy-containing visits

Cycle 1 lasts 42 days: nivolumab, ipilimumab, paclitaxel and carboplatin on Day 1; nivolumab, paclitaxel and carboplatin on Day 22. These are the only two planned chemotherapy administrations. From Cycle 2, nivolumab is given on Days 1 and 22 and ipilimumab on Day 1. Paclitaxel premedication includes corticosteroid and antihistamine treatment, with H2-blocker decisions made by the team. The source allows consultant-directed changes; ask for a written plan for each of the two chemotherapy visits. The team must reconcile paclitaxel allergy-prevention steroids with anti-sickness steroids and the aprepitant interaction into one prescription. Do not add courses or take leftover steroids yourself. Use dedicated carboplatin AUC≥4 NCIS row; no routine olanzapine in that row. Myalgia/arthralgia may need an individual analgesia plan. Both immune medicines can inflame healthy organs during treatment or after it stops. Keep your immune alert card and report new symptoms promptly, including for at least five months after the last dose.

2 Cycle 2 onwards · Day 1 nivolumab + ipilimumab

Both immune medicines can inflame healthy organs during treatment or after it stops. Keep your immune alert card and report new symptoms promptly, including for at least five months after the last dose. Chemotherapy is complete; review and stop induction prophylaxis rather than automatically repeating it.

Treatment effects

Bowel, lungs and skin

New diarrhoea, tummy pain, blood/mucus in stool, cough, breathlessness, blistering or peeling skin need prompt assessment.

Hormones, heart, muscles and nerves

Unusual weakness, headache, dizziness, marked thirst, frequent urination, chest pain, palpitations, drooping eyelids or difficulty swallowing/breathing need urgent advice. Several organs can be inflamed together.

Chemotherapy effects

Paclitaxel/carboplatin can cause low blood counts, infection, hair loss, neuropathy, muscle pain, nausea and infusion reactions. Report symptoms before another dose.

Support medicines and self-care

When unwell

Follow your oncology emergency instructions for fever, shivering or sudden illness even with a normal temperature. Severe breathing difficulty, chest pain, collapse or confusion needs emergency care.

After treatment and daily life

Reproductive advice

Discuss pregnancy, breastfeeding, fertility and contraception before treatment. Effective contraception is needed for at least five months after the last nivolumab dose; ask about your individual plan.

Living with treatment

Ask for help with fatigue, appetite, pain, breathlessness, work and emotional concerns. Follow-up continues for unresolved effects and any long-term hormone replacement.

Questions worth asking

What if I miss a support medicine?

Ask your oncology team or pharmacist for advice; do not double doses or combine steroid courses yourself.

How is benefit checked?

Your consultant sets the scan plan and reviews symptoms, results and priorities. Side effects do not reliably show cancer response.

Can I decide from this guide whether treatment is safe today?

No. The team must assess symptoms, blood results, previous reactions and the prescribing protocol.

Try explaining the plan back: Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

Nivolumab

- Immune inflammation in multiple organs
- Delayed effects after treatment stops

Ipilimumab

- Immune bowel, liver, skin and hormone inflammation
- Other serious organ inflammation

Paclitaxel

- Low blood counts and infection
- Peripheral neuropathy and hair loss
- Allergic reactions
- Muscle and joint pain

Carboplatin

- Nausea and vomiting
- Kidney, hearing and nerve effects

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cycle 1 Days 1 and 22 · chemotherapy-containing visits

Active medicines: nivolumab (Days 1 and 22) + ipilimumab (Day 1 only) + paclitaxel (Days 1 and 22) + carboplatin (Days 1 and 22)

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally on treatment day; 80 mg orally on each of the following two days	Repeat the three-day course separately around Cycle 1 Day 1 and Day 22
Ondansetron	16 mg orally	Each chemotherapy treatment day Review QT risk, interactions and individual dose choice.

Medicine	Dose / route	When
Antiemetic dexamethasone reference	12 mg orally on treatment day; 8 mg orally daily for the next three days	Repeat around both chemotherapy visits The team must reconcile paclitaxel allergy-prevention steroids with anti-sickness steroids and the aprepitant interaction into one prescription. Do not add courses or take leftover steroids yourself.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS high-risk row does not define an end day or PRN qualifier; prescriber must specify duration, suitability and interaction risks.
Paclitaxel dexamethasone premedication	Source reference 20 mg oral or intravenous (IV)	Oral approximately 6 and 12 hours before paclitaxel, or IV 30 minutes before Reconcile with antiemetic steroid; source advises considering 12 mg on treatment day when aprepitant is added. Consultant may reduce/omit only under explicit guidance.
Chlorphenamine	10 mg IV	30 minutes before paclitaxel
Famotidine	20 mg IV	30 minutes before paclitaxel Omission requires consultant guidance. Source cautions particularly with every-three-week paclitaxel; monitor closely for hypersensitivity.

Additional phase-specific support

- Cycle 1 lasts 42 days: nivolumab, ipilimumab, paclitaxel and carboplatin on Day 1; nivolumab, paclitaxel and carboplatin on Day 22. These are the only two planned chemotherapy administrations. From Cycle 2, nivolumab is given on Days 1 and 22 and ipilimumab on Day 1.
- Paclitaxel premedication includes corticosteroid and antihistamine treatment, with H2-blocker decisions made by the team. The source allows consultant-directed changes; ask for a written plan for each of the two chemotherapy visits.
- The team must reconcile paclitaxel allergy-prevention steroids with anti-sickness steroids and the aprepitant interaction into one prescription. Do not add courses or take leftover steroids yourself.
- Use dedicated carboplatin AUC≥4 NCIS row; no routine olanzapine in that row. Myalgia/arthralgia may need an individual analgesia plan.
- Both immune medicines can inflame healthy organs during treatment or after it stops. Keep your immune alert card and report new symptoms promptly, including for at least five months after the last dose.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Cycle 2 onwards · Day 1 nivolumab + ipilimumab

Active medicines: nivolumab + ipilimumab

Emetogenic risk: low

Medicine	Dose / route	When
Ondansetron	8 mg orally	Day 1 before treatment, if following the NCIS low-risk default Individualise/local policy. No automatic antiemetic corticosteroid course.
Metoclopramide	10 mg orally three times daily as needed	Subsequent days as prescribed Specify duration and check contraindications/interactions.

Additional phase-specific support

- Both immune medicines can inflame healthy organs during treatment or after it stops. Keep your immune alert card and report new symptoms promptly, including for at least five months after the last dose.
- Chemotherapy is complete; review and stop induction prophylaxis rather than automatically repeating it.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Cycle 2 onwards · Day 22 nivolumab alone

Active medicines: nivolumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Both immune medicines can inflame healthy organs during treatment or after it stops. Keep your immune alert card and report new symptoms promptly, including for at least five months after the last dose.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

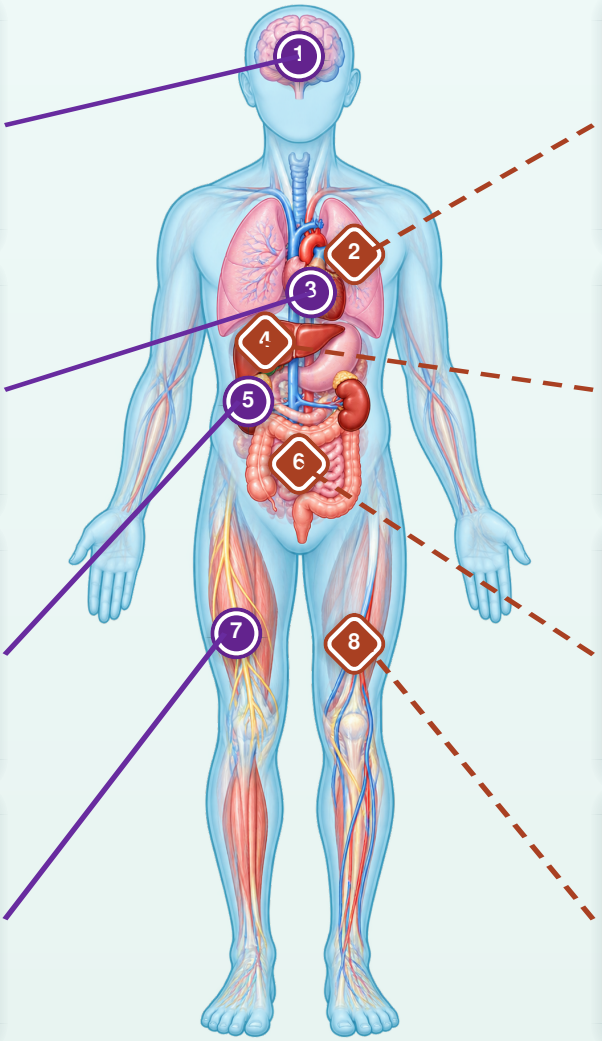
Nivolumab, ipilimumab, paclitaxel and carboplatin

Cycle 1 lasts 42 days: nivolumab, ipilimumab, paclitaxel and carboplatin on Day 1; nivolumab, paclitaxel and carboplatin on Day 22. These are the only two planned chemotherapy administrations. From Cycle 2, nivolumab is given on Days 1 and 22 and ipilimumab on Day 1. Continue while appropriate until progression, unacceptable toxicity or up to 24 months without progression. Your team reviews the plan and may delay or stop treatment.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Hormones / brain

Nivolumab / ipilimumab

Unusual weakness, headache, dizziness, thirst or passing much more urine.

2 Lungs

Nivolumab / ipilimumab

New cough or breathlessness needs prompt assessment; infection or cancer can cause similar symptoms.

3 Heart / muscles

Nivolumab / ipilimumab

Chest pain, racing heartbeat, fainting or new muscle weakness needs urgent assessment.

4 Liver

Nivolumab / ipilimumab

Yellow eyes/skin, dark urine or abnormal tests can signal inflammation.

5 Kidneys

Carboplatin / immune medicines

Kidney tests matter; report less urine, swelling or difficulty drinking.

6 Bowel / mouth

Paclitaxel / immune medicines

Report new diarrhoea, pain, blood/ mucus or mouth ulcers; do not assume ordinary chemotherapy upset.

7 Skin / nerves / muscles

Paclitaxel / immune medicines

Report spreading rash, blistering, numbness, weakness or difficulty swallowing.

8 Blood / infection

Paclitaxel / carboplatin

Fever, shivers, sudden illness or unusual bleeding need immediate oncology advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Follow your oncology team's emergency instructions for fever, shivering or sudden illness, including with normal temperature. Report new breathlessness/cough, diarrhoea or abdominal pain, reduced urine, severe rash, unusual weakness, headache or marked thirst promptly. Severe chest pain, collapse, confusion or major breathing difficulty needs emergency care.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00712 v3a · official regimen**
   https://healthservice.hse.ie/documents/6879/712_V3a_Nivolumab_ipili_plus_PACLitaxel-CARBOplatin.pdf
2. **CheckMate 9LA — final six-year analysis (2025)**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC12166824/
3. **EMA Opdivo intravenous (IV) product information**
   https://www.ema.europa.eu/en/documents/product-information/opdivo-epar-product-information_en.pdf
4. **EMA Yervoy product information**
   https://www.ema.europa.eu/en/documents/product-information/yervoy-epar-product-information_en.pdf
5. **NCCP NCIS Medical Oncology antiemetics V8**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf